



Patient Profile

Name: Mr. PL94

Age: 35 years

Gender: Male

Summary of Results

The laboratory evaluation reveals isolated elevations in total and direct bilirubin and mild AST elevation, accompanied by a low alkaline phosphatase level. The predictive model classifies the overall profile as **normal**; however, specific derangements warrant focused attention.

Detailed Test Explanations

Test	Normal Range	Patient Result	Interpretation
Bilirubin Total	0.0–1.2 mg/dL	14 mg/dL (High)	Significantly elevated total bilirubin suggests cholestasis, hepatocellular injury, or hemolysis.
Bilirubin Direct	0.0–0.2 mg/dL	1.2 mg/dL (High)	Elevated conjugated bilirubin indicates hepatocellular or obstructive biliary pathology.
Serum Bilirubin Indirect	0.00–0.90 mg/dL	0.20 mg/dL	Within normal limits; low indirect fraction relative to the total points to a primarily conjugated hyperbilirubinemia.
SGOT / AST	0–33 U/L	36 U/L (High)	Mild elevation of AST may reflect hepatic cell injury or muscle involvement.
SGPT / ALT	0–41 U/L	35 U/L	Within normal limits; preserved ALT reduces likelihood of significant hepatocellular damage.
AST / ALT Ratio	–	1.03	Near unity; suggests non-alcoholic fatty liver disease or early hepatic injury rather than alcoholic hepatitis.
Alkaline Phosphatase (ALP)	40–129 U/L	25 U/L (Low)	Low ALP may represent malnutrition, hypophosphatemia, or a laboratory error; isolated low value requires correlation.
Total Protein	6.4–8.3 g/dL	6.9 g/dL	Normal.
Albumin	3.5–4.8 g/dL	4.6 g/dL	Normal.
Globulin	1.9–3.7 g/dL	2.3 g/dL	Normal.

Test	Normal Range	Patient Result	Interpretation
Albumin/Globulin (A/G) Ratio	1.0–2.1	2.0	Normal.

Overall Interpretation

The profile demonstrates marked hyperbilirubinemia with a predominance of the conjugated fraction, accompanied by a modest rise in AST and a low alkaline phosphatase. These findings suggest a hepatic or biliary process, possibly cholestatic in nature, despite the overall “normal” classification by the predictive algorithm. The absence of significant ALT elevation and a balanced AST/ALT ratio reduces the likelihood of acute viral hepatitis or alcoholic liver injury.

Recommendations

- **Repeat Liver Function Tests** within 4–6 weeks to assess trend, particularly bilirubin and ALP.
- **Imaging:** abdominal ultrasound to evaluate biliary tract obstruction or hepatic parenchymal changes.
- **Clinical correlation:** inquire about jaundice, pruritus, dark urine, or pale stools; review medication history for hepatotoxic agents.
- **Lifestyle:** maintain adequate hydration, balanced diet rich in antioxidants, avoid alcohol and hepatotoxic substances.
- **Follow-up:** consultation with a hepatologist if bilirubin remains elevated or imaging suggests pathology.

Disclaimer

This report is for informational purposes only and does not replace professional medical evaluation or diagnosis. Consult a qualified healthcare provider for personalized medical advice.